

E BOOK

Addiction Disorders



Tulasi Healthcare

Mental Health Establishments and Rehabilitation
Facilities for Psychiatric Disorders and Substance Abuse



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INTRODUCTION

What is Addiction?

Addiction is a relapsing , remitting disorder which can be managed with help of medication and psychotherapy. Individuals who are addicted take drugs or participate in repetitive behaviors that they frequently maintain in spite of knowing the negative outcomes or consequences. It is caused by a complex interplay between genetic, environmental, and neurochemical experiences.

The following are the main signs of substance addiction:

- falling grades or trouble in school;
- declining performance at work
- relationship problems including frequent fights with family members, friends or people who express a disapproval against addiction in general.
- the inability to give up a substance even when it may be contributing to personal or health concerns.
- loss of interest or lack of vigor in regular day to day tasks
- significant physical changes, such as weight loss and a clear disregard for personal hygiene
- taking increasing quantity of the substance to achieve a sense of high
- using the substance despite knowing the adverse consequences

People may seemingly experience apparent benefits from drugs/substance when they take them for the first time, and they may also think they have control over their use. But a person's life can be quickly taken over by the substance. If the substance usage persists over time, other enjoyable activities eventually lose their appeal, and the user eventually has to use the drug in order to feel a sense of normality. Then, despite knowing the fact that using drugs poses serious risks to both themselves and their, family, they could become compulsive seekers and users.

Most of the time, consuming the substance is a voluntary choice. But sustained use can cause a person's capacity for self-control to deteriorate significantly; this is the defining feature of addiction. Studies have shown that due to continuous use of the substance, certain areas of the brain get deeply affected which in turn are responsible for making decisions, judgment, memory, learning, and behavior regulation.

Addiction susceptibility varies from person to person, just like with any other disease, and no single characteristic can predict a person's likelihood of developing a drug addiction. The likelihood that drug usage will result in misuse and addiction increases with an individual's exposure to certain risk factors. Conversely, protective factors lessen an individual's likelihood of becoming addicted.

Behavioural Addiction

Any recurrent and persistent behavior that negatively impacts a person's psychological, physical, interpersonal, and financial well-being is classified as behavioral addiction.

Behavioral addiction symptoms include:

- hiding or downplaying the seriousness of the problem
- relying on the behavior as a means of coping with different emotions
- performing the behavior most of the time
- failing in your efforts to refrain from engaging in the addictive behavior
- continuing the behavior despite knowing the negative consequences

Genetics, co-occurring psychological issues, previous trauma or abandonment issues, and specific personality features are indicators of risk for behavioral addiction. It is commonly accepted that those with a family history of addiction or psychological illnesses are more susceptible to behavioral addiction.

Behavioral addiction may also be more likely to develop in someone who already has a diagnosis of a mental health illness because addictive disorders and mental health conditions frequently coexist. Adverse experiences, such as maltreatment or trauma, particularly during early childhood, might also contribute to the later emergence of addictive behaviors.

Finally, impulsivity and a risk taking behavior are two characteristics of personality that may increase a person's likelihood of engaging in harmful behaviors.

Different types of Behavioural Addiction are:

Gambling Addiction

A person with an addiction to gambling experiences strong impulses to continue gambling despite the fact that it interferes with their day-to-day activities. An excessive preoccupation with engaging in gambling, spending increasing amounts of time and money to experience the same thrill, failing attempts to reduce or quit, and lying to loved ones to hide and deny their problematic activity are all signs of compulsive gambling.

Some of the symptoms of the disorder include relationship problems, financial hardships, feeling of guilt regarding their behavior, poor performance at work or school, and self-harming thoughts and behaviors.

Pathological gambling symptoms include:

- Difficulty in controlling one's gambling behavior and futile attempts to quit; • Financial problems resulting from gambling, including the need to gamble with increasing amounts of money to achieve desired excitement.
- Negative repercussions such as lying about the extent of one's involvement with gambling and upsetting family and employers.
- Preoccupation with gambling

Internet Addiction

Frequent and compulsive internet use that interferes with day-to-day functioning is the hallmark of internet addiction, a type of behavioral addiction. The incapacity to limit the use of internet-accessible devices despite adverse effects, recurrent failure in attempts to cut back on internet use, and failure to finish tasks at home, work, or school are some of the symptoms of internet addiction.

Genetics, co-occurring mental health issues like anxiety and depressive disorders are the reasons of problematic internet use. Weight gain, dropped projects, unfulfilled responsibilities in many areas of life are among the short-term consequences of internet addiction. Long-term impacts include visual issues, neck and back discomfort, and social disengagement.

Porn Addiction

An inability to control the urge to watch porn or use other pornographic materials is known as porn addiction. The symptoms include desires to watch more porn, binge-watching porn for longer than planned, losing interest in a relationship because of erroneous sex expectations, and repeatedly failing to reduce porn consumption. Procrastination, a sense of boredom, a dissatisfied personal partner, and social issues are some of the short-term consequences of a porn problem.

Sex Addiction

Being preoccupied with participating in sexual activities even when there are negative repercussions is known as "sex addiction." Indulging in sexual acts in unsuitable settings, being preoccupied with having sex, being unable to manage one's sexual cravings, and using sexual acts as an outlet for unpleasant feelings are all signs of sex addiction. The illness is also associated with certain risk factors, such as gender, substance misuse history, and past experiences of sexual or physical assault. Sex addiction can seriously harm a person's relationships and health, much like other behavior addictions. Impaired emotional regulation, incapacity to finish household chores, and decreased work productivity are some of the immediate consequences of sex addiction.

The Science Behind Addiction

Understanding the science behind addiction entails taking a multidimensional approach. Addiction is a complex disease. Everyone who consumes substances does not get addicted to them. An individual's genetic factor, personality and environment may determine if they may get addicted to substances.

But an individual's biological endowment is one of the most important factors that dictate the onset of addiction. Genetics plays a substantial role in the development and progress of the disease of "Addiction". According to research, heritability is responsible for 40-60% of an individual's vulnerability to developing this disease.

Role of the brain in addictive disorders

The brain is an intricate organ in the human body. Our brain consists of billions of cells known as neurons. Neurons organize into networks. Our brain uses these networks to transmit information and manage the information that transfers from the body to the brain and the messages that the brain sends to the rest of our body. Moreover, the brain releases and absorbs certain chemicals known as neurotransmitters.

Feeling pleasure is how a healthy brain recognizes and reinforces beneficial behaviours such as eating, socializing, and sex. Our brains are wired to increase the likelihood of us repeating pleasurable activities. The neurotransmitter dopamine is integral to this process. Dopamine is responsible for activating the reward circuit. The difference between regular rewards and drug rewards can be compared to the difference between someone tapping on a surface with their finger and someone beating a drum. The brain of an addict adapts by producing fewer neurotransmitters in the reward system or by reducing the number of receptors that receive the signals. As a result, a person's ability to experience pleasure from naturally rewarding activities also decreases. A person who abuses drugs feels flat, unmotivated, lifeless and/or depressed and unable to enjoy things that used to be pleasurable. Now the person must keep using drugs to get even a normal reward, which only makes the problem worse, like a vicious cycle. In addition, a person often needs to take larger amounts of the drug to develop tolerance.

Addiction is a disease, not a moral failure

Addiction is a disease, not a weakness in a person. Despite knowing that drugs are bad for them, people with addiction struggle to limit their drug usage or repetitive behaviour. It takes more than just using willpower to avoid drug temptation or overcome a behavioural addiction. Along with various forms of therapy, taking medication to help with withdrawal symptoms and cravings may be part of the recovery process.

Social stigma has long surrounded addiction. Many people believe that the severity of these problems stems from a lack of self-control and individual behavioral choices.

It is commonly acknowledged that substance abuse is a complicated medical disorder that affects the brain, changes behavior, and can result in chronic addiction. This addiction is the product of a confluence of behavioral, environmental, and genetic elements rather than a moral failure or a lack of willpower. Therefore, to ensure that persons who are battling with addiction receive appropriate and compassionate care, treatment should be seen as a medical requirement in order to ensure that individuals can live a healthy life.

CHAPTER I

TRIGGERS

Triggers can occur for people in recovery and triggers often lead to craving.

Types of triggers–

Environmental triggers:

- Being in particular places (such as pubs or neighborhoods linked to previous usage);
- Associating with those who engage in drug or alcohol use;
- Experiencing stress at work or at school, or financial difficulties
- Dealing with disputes like divorce and breakups
- Being a part of events like holidays, festivities, anniversaries, and vacations;
- Observing advertisements for drugs on TV shows, movies, or social media.

Being conscious of what's happening around is the greatest method to prevent environmental triggers. Limiting your interactions with some people is important since being near them can trigger a relapse.

Emotional triggers have the potential to cause relapses in recovery. Some typical instances of emotional triggers are as follows:

- Sense of isolation or loneliness;
- Dealing with critical comments or rejections from others;
- Feeling helpless in specific circumstances;
- Having excessive self-criticism due to past faults;
- A lower sense of self-worth;
- Feeling intense emotions, such joy or rage;
- Feeling extreme anxiety, fear, and doubt regarding sobriety;

Engaging in healthy habits like exercise and meditation is crucial in practicing self-care since it lowers the likelihood of relapsing due to emotional triggers for addiction. Constructing a reliable support network is also beneficial for recovery.

Behavioral triggers

Typical behavioral triggers include things like:

- Spending excessive amounts of time idle without engaging in worthwhile hobbies or activities
- Failing to indulge in eating healthy food and inability to maintain proper sleeping schedules;
- Ignoring meetings or therapy sessions
- Lacking a strategy for handling stressful circumstances or triggers

Making a new schedule devoid of activities related to substance use is one of the most crucial strategies for avoiding behavioral triggers. Actively occupying oneself with tasks like writing, reading, working out, or taking a stroll are all excellent ways to pass the time when you have some spare time.

CHAPTER II

RISK FACTORS

Stress and Vulnerability

High levels of unrelieved stress have been shown in numerous studies to alter brain chemistry in a way that increases the risk of mental health illness. Trauma or other difficulties that we have not fully handled in the present might also be the cause of stress. If this model were to be applied to addiction, it would imply that some individuals have a heightened genetic or biological vulnerability to the brain's addiction process from birth and that, for them, environmental factors like drug exposure and stressful or traumatic experiences, combined with a lack of control over behaviour, could be the catalyst for addiction.

Biological factors

There is compelling evidence from certain studies that genetics has a role in addiction vulnerability. According to research, the probability of an identical twin developing an addiction is significantly higher than that of a fraternal twin if one twin develops an addiction. Having a family member who suffers from addiction may increase your own risk of substance abuse problems. You may be more susceptible to addiction due to your genetic composition. Your body's reaction to substances and the likelihood that you may become addicted can be influenced by factors such as age, sex, race, and mental health.

Family and Home Environment

Since genetics alone cannot fully account for addiction, environmental factors are likely to play a significant role. One clear environmental factor is the substance's accessibility. For example, people who are raised in a strict society where alcohol is not consumed are less likely to develop an alcohol problem as adults, while those who reside in a remote place where heroin is difficult to obtain are less likely to develop a heroin addiction. However, a young person who lives in an area where drugs are easily accessible at the corner store and has minimal parental supervision is far more vulnerable.

Individual Characteristics and Personality

According to research, there is a particular personality type that is frequently—though not always—found in those who struggle with addiction. Many addicts claim to have experienced high levels of anxiety as children. They frequently had feelings of social awkwardness, depression, sensitivity, anxiety, inadequacy, dislocation, and low self-worth and self-esteem. These emotions frequently appear to have developed on their own, even if they may be linked to a challenging childhood.

Protective factors

- Adaptability to change and adverse situations;
- Engaging in religious practices or spirituality
- Confidence in one self and secure attachment style
- Support from family and friends
- Good academic/occupational performance

CHAPTER III

ADDICTION AS A FAMILY DISEASE

Addiction is a disease that significantly impacts the family of an addict. Everyone's world is turned upside down when an addict begins to alter in behaviour and personality drastically and mysteriously. Family members now must cope with someone who may be unpredictable, self centered, argumentative, untrustworthy, and distant. Previously, family members who relied on the addict for everything from income to emotional support to household tasks, may not be able to accept this change. Many families feel that their experience is unique and that no one else can relate to it because addiction is stigmatized and many individuals are secretive about it. However, the severe disruption of families brought on by addiction is all too prevalent. It is difficult to envision a family responding in any other manner.

Emotional toll on loved ones

Uncertainty

There is a lot of confusion and anxiety when family members first get to know that their loved one is suffering from addiction. Addicts can occasionally be quite skilled at concealing their substance use. The family typically initially observes behavioural and personality changes in their loved ones. The individual becomes more defensive, aloof, impatient, irritated, or short-tempered. Addicts tend to disappear, sleep in late, or remain out late. Initially, the family does not know that a substance is the root cause of this issue. In some situations, the family knows that the member is abusing substances more than usual, but they are hesitant to declare them to be an addict.

Stress

Everybody in the family endures tension brought on by addiction. Everyone in the family must adapt to the changing dynamics, which is a very stressful situation. Furthermore, there may be a great deal of fear and anxiety regarding the person's future and what to do next.

The addict's actions often make people feel more stressed. Addicts may behave carelessly, start arguments, destroy objects, breach commitments, and so forth. Many individuals become accustomed to the addict's actions by being cautious and attempting to avoid starting a fight.

Anger

Families who have loved ones who are addicts are upset about how their lives have changed and the fact that the addict is acting carelessly. Additionally, when an addict gets defensive or agitated, they frequently say hurtful things to their family members, which can make them angry. Unfortunately, rage breeds more rage. Conflicts can become a common occurrence as family members let their anger out on the addict, who usually reacts in the same manner. It can be hard to remember that an addict has a disease while they are acting angrily in the heat of the moment.

Mistrust and Hurt

The family's belief that the addict has abandoned them or betrayed them is a significant contributing factor that hurts them. The substance abuser acts as if they do not care for their family. The addict does not listen to them, doesn't do anything for them, and won't alter their behaviour to make their family feel better.

Moreover, the addicts keep lying and manipulating their families. As a result, the families often stop trusting them. They do not want to believe them when they make excuses, or promise to do better, because they are let down so many times by the addict.

Guilt and Self Doubt

Family members often feel guilty since they do not find any other explanation for the actions of the addict. Furthermore, the addict often plays on and encourages these feelings because it deflects blame and allows them to avoid facing the truth. Even when it is evident that the addict has a problem, the family members sometimes question their understanding of what is going on. This happens because the addict engages in denial, suppression, making excuses, outright lying, and other behaviours to undermine the perceptions of their family members.

Obsession

The family tries to understand the situation at hand. They think a lot about their altercations with the addict, and they also try to dodge confrontations. Eventually, the thoughts about the addict crowd everything else in the family's lives, interfering with their work, school, friendships, leisure, habits, and overall health. In other words, just as addiction takes over the lives of addicts, it eventually takes over the whole family.

Isolation and Grief

Families of addicts are often cut off from friends, relatives and normal social and leisure activities. Addiction often creates shame and confusion, and because an addict's behaviour can be so unpredictable, families often stop inviting people into their homes for fear that visitors will discover their "family secret". The family also experiences grief and sorrow. Even when the addict is alive and well, family members often mourn the loss of the person and the loss of the relationship they had with them.



CHAPTER IV

THE SCIENCE BEHIND ADDICTION

Drug exposure affects our reward systems, our capacity to remember, and ability to make decisions, which can result in an unmanageable pattern of cravings.

Addiction is a hereditary trait for some people, but it also includes basic brain functioning. But not everyone develops an addiction.

The repeated use of drugs alters the way the brain works. Drug usage eventually turns from voluntary or recreational use to a regular/compulsive use. Using drugs is no more in control of the individual; it no longer remains a choice.

Researchers have discovered that addiction takes over and even damages important brain areas designed to aid in our survival.

Dopamine is a pleasure producing neurotransmitter that the brain generates when someone uses drugs. As a result, the individual who uses the drug feels happy or elevated. The brain gets dependent on the pleasurable feeling when it goes through this frequently. This eventually becomes a circuit and continuous use of drug becomes a topmost priority for the individual.

A functioning brain prompts our body to respond rapidly with dread or alarm when we are in danger, helping us to escape the situation. When determining if an action is worthwhile, the frontal lobes of our brain can assist us. But the typical integration of important brain functions might start to act against us when we start to become addicted to a substance. Alcohol and drugs have the power to take over our brain's pleasure and reward pathways, making us crave more and more of them.

Stressful emotions like worry, anger, and uneasiness, which accompany drug withdrawal after the high wears off and encourage relapse, are influenced by an elongated amygdala. The more drugs used, the more sensitive this circuit gets. Circuits that sense emotional danger may go into overdrive as a result of addiction, causing us to experience worry and anxiety even when we're not consuming substances. At this point, rather than using drugs or alcohol for their enjoyable benefits, people frequently use them to avoid feeling horrible.

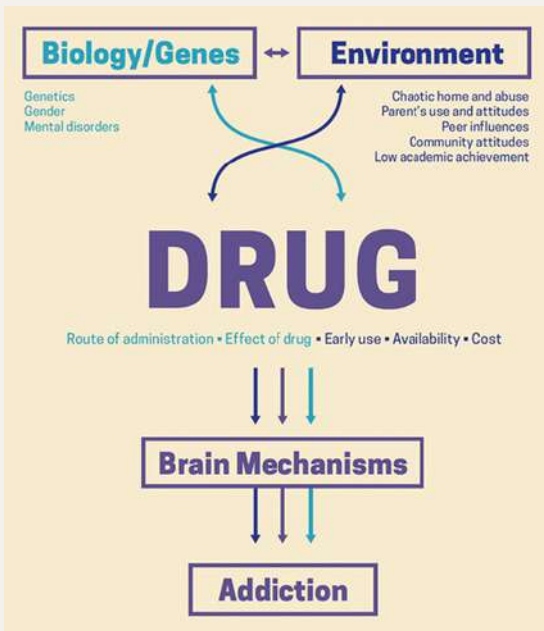
Substance abuse on a regular basis can harm the frontal lobe of the brain, which is crucial for making decisions. The prefrontal cortex is the exact place that ought to assist you in identifying the negative effects of abusing substances that are addictive. People who experience frontal cortex dysfunction are unable to decide whether to stop using drugs, even when they are aware of the potentially fatal consequences of doing so.

CHAPTER IV

Affected simultaneously are the areas of the brain linked to reasoning, making decisions, learning, and control over oneself as well as the system for rewarding behavior. Due to these changes in the brain, substance users find it more difficult to stop using drugs, as the individual is unable to make correct decisions or keep their impulses under control.

The stress network is an additional focus for addiction treatment programs. Stress can change the manner in which the brain functions, especially in terms of how it weighs the effects of decisions. Stress shuts down the prefrontal cortex region that is engaged in deliberative thought due to which the brain is unable to take the correct decisions.

Brains of teenagers are not fully developed, especially the frontal lobe that aid in impulse control and risk assessment, make them more susceptible to potential addiction. Adolescent brains' pleasure circuits are also hyperactive, which makes using drugs and alcohol even more pleasurable and rewarding. Hence, leading to addiction.



Source: NIDA. 2022, March 22. Drugs and the Brain. Retrieved from <https://nida.nih.gov/publications/drugs-brains-behavior-science-addiction/drugs-brain> on 2024, March 10

CHAPTER V

TREATMENT

Overcoming addiction requires receiving proper treatment. Recognizing that addiction has become a problem in one's life and is negatively affecting one's quality of life is an initial step towards recovery.

Treatment starts with assessment and observation followed by a combination of pharmacological and non pharmacological treatment modalities. Several treatment modalities are available for the treatment of addiction and depends on different factors such as the frequency and intensity of use, and the repercussions on the individual. The majority of people obtain a mix of treatment modalities, as there are numerous options accessible.

Detoxification

Typically, the first stage of treatment is detoxification. Detoxification means minimizing withdrawal symptoms with a replacement therapy in the form of pharmaceutical agents.

Psychotherapy

Psychotherapies assist patients in drug addiction in changing their thought pattern and drug-related behaviors. Patients are therefore more equipped to manage stressful circumstances and other triggers that could lead to another relapse. Additionally, behavioral therapy can help patients stay in treatment longer and improve the efficacy of drugs. Different therapies are available for the same-

Cognitive Behavioural Therapy (CBT) assists patients in identifying, avoiding, and managing the circumstances that lead to drug use. This helps to change or replace maladaptive thoughts with more healthy and adaptive thoughts.

Motivational Enhancement Therapy (MET) employs techniques to maximize individuals' willingness to modify their behavior, understand the discrepancies caused in their life due to excessive consumption of substance and take treatment in order to bring a change.

Family therapy addresses factors that influence drug use patterns and enhances general family functioning for individuals with substance use problems, particularly young people and their families.



Inpatient Rehabilitation

Inpatient rehabilitation, usually referred to as residential treatment, provides care around-the-clock. Rehabilitation facilities are typically created by a multidisciplinary team of medical specialists, including psychiatrists, physicians, nurses, therapists, and counselors. They provide comprehensive recovery solutions by offering multifaceted treatment strategies that address addiction and mental health concerns. Individuals undergo medically supervised detoxification, and therapies address underlying issues and assist in creating healthy coping mechanisms. Focused recovery is possible because of the reduced stimuli in the structured environment. Rehabilitation offers peer support, knowledge about stressors and relapse prevention. The goal is to assist individuals in returning to their regular lives and enhancing their quality of life.

Medication

During detoxification, people most frequently take medications to treat their withdrawal symptoms. Depending on the drug to which the patient is addicted, the medication will change. Furthermore, medication may also be used to address co-occurring mental health issues, if any. Extended usage of pharmaceuticals lessens cravings and keeps people from relapsing—using substances again after they have overcome their addiction.



Addiction: a paradoxical force offering solace, exacting a heavy toll. Driven by desire, it traps the mind, rendering the afflicted powerless. Beneath pleasure lies suffering—a perpetual struggle for control.

Dr. Gorav Gupta



CHAPTER VI

RELAPSE

A "relapse" occurs when a recovering addict resumes using the substance. Relapses are possible at any moment, in the first year of recovery, and especially in the initial weeks and months. This happens because the addicts are only now learning how to build a new life centered around anything other than substance abuse, and their brains are still recovering from the changes caused by the disease. Nevertheless, addiction is a chronic disorder that lasts a lifetime, and relapse can happen even after a long period of abstinence. Each recovering addict may experience relapse for a different reason, and there is rarely just one particular trigger, but several interrelated circumstances come together to induce a relapse.

Triggers associated with Relapse

Stress

An addict abuses substances to cope with tension and worry. Stress is a trigger because it makes the recovering addict want to take the substance that seemed to have helped so much in the past. Additionally, especially in the early stages of recovery, the addict is making a lot of effort to deal with life's challenges while remaining resolute in their decision to abstain from drugs. Stress makes it more difficult to deal with life, which makes it more difficult to stay true to one's goals.

Being in contact with the substance

A powerful trigger can be coming into contact with the substance of choice used by the addict (such as an alcoholic in recovery entering a bar). Exposure to associated objects, such as empty beer bottles or drug paraphernalia, and drug exposure leads to temptation. Research has demonstrated that the mere sight of an image linked to drug use, like a syringe or a pile of white powder, can trigger an abrupt release of dopamine in the joy center in the brain of an addict. In essence, the exposure results in a mild high similar to the drug's actual effects. This may restart the brain's biological process of addiction.

Cues from the Environment

"Environmental cues" is a word used to describe exposure to other things that addicts subconsciously link to their previous usage rather than the substance itself. It could be a reference to a space where they used to abuse the substance of choice, a certain bar, a location where they used to buy drugs, friends they used to hang out with, particular genres of music, etc.

What to expect in recovery

Recovery of the patient

Addiction results in anatomical alterations in the prefrontal cortex and pleasure areas of the brain. Even after a detox and overcoming withdrawal symptoms, those modifications in the brain persist. They do not disappear. The brain is capable of self-healing, but it takes time. The prefrontal cortex of the brain typically takes three months to start reverting to a somewhat normal condition.

Until that happens, there is still an excessive and unnatural bias in favor of substance use when making decisions. The hardest and most likely time to relapse in sobriety is often within the first three months of recovery. Addicts are incredibly vulnerable. Even though they might not be utilizing substances actively, their brain urges them to do so. It takes significantly longer for the pleasure-center region of the brain to heal.

The addict recovers most of their ability to make decisions after three months of sobriety, but their emotional cravings to use the substance persist. The addict who is not getting a drug-related dopamine fix frequently experiences a happiness deficit because dopamine is still not being released correctly. Anxiety, impatience, mood fluctuations, fatigue, sadness, difficulty concentrating, and sleep issues are among the common emotional symptoms.

Recovery of the family members

For family members, the reality of an early recovery can be quite challenging. The family members often perceive addicts as self-centered, prioritizing their own needs over those of others, and concentrating all of their attention on their substance use. Family members are frequently deeply disappointed. They still have not succeeded in their quest to reconcile with the person they once knew. Rather, they might be cohabiting with someone who exhibits mood swings, irritability, anger, self-absorption, and is likely unable to assume all the obligations of their previous existence. Anger, hurt, preoccupation, and all the other emotions that family members are likely to have experienced during the addict's active addiction are still present. Once the addict gets back home, families frequently hope that the addicts would accept the grief and the suffering that their families have endured, as well as offer an apology for their actions. They believe that, the addict will notice them and make amends for what they went through. Unfortunately, most addicts will only gradually acquire the ability to attend to the emotional needs of others.

How to support your patient during recovery

Actively listen to them and follow up with them

It is more crucial than ever to check in on your loved ones and see how they are feeling and doing. Everyone needs to feel heard and wanted. When your loved one is sharing something with you, make sure you are attentive and involved, show that you are paying attention to them by expressing worry and making sure you understand them. Offer your affirmation of their feelings, even if you disagree.

Lessen tension and pointless disputes

Household strain can lead to onset of relapse in a recovering addict. Contribute to the growth of the recovering addict with constructive support by engaging in open discourse and healthy communication. Aim to avoid needless or undesirable conversation subjects and instead make an effort to spend quality, meaningful time together.

Promote healthy behaviors

Demonstrate your support by leading a healthy lifestyle yourself, whether that means maintaining open lines of communication, getting enough sleep, exercising, eating well, or giving up alcohol, drugs, and smoking. Beyond just helping your loved one, this will also benefit you by enhancing your physical, mental, and spiritual well-being and creating new memories.

Avoid passing judgment and criticism

Addicts and alcoholics in recovery frequently struggled with a tremendous lot of shame and remorse in the past and may still harbor unfavorable self-perceptions. It is essential not to judge them. Accept, love, and value people for who they are rather than passing judgments on them.

Give them encouragement and help

Express your pride in your loved one's recovery to show them your encouragement and support. Family's approval, love and encouragement can make a huge impact during recovery. Encourage them to participate in therapy, addiction recovery programs, mutual aid organizations, and other recovery-related activities.

Show patience

Recognize that nobody transforms into someone else overnight.

Even after entering treatment, a person may continue to participate in harmful habits or make bad choices. Abstaining from substance abuse involves a lot more than just quitting substance use, improvement and recovery takes time and patience. Relapses and other setbacks do occur. If they do, try to be patient with yourself and your loved one, and try expressing your love, care, and support for them.

Establish reasonable boundaries

Bad boundaries are frequently established during active addiction and may persist following recovery. Everyone must assess any problematic behaviors they may be engaging in, such as codependency or enabling.

Reduce the triggers in the environment

The family should forego taking substances themselves, and should not keep them in their homes. If you do consume alcohol, be careful not to do so while your loved one is in recovery. If you are taking prescription drugs, keep them hidden from your recovering loved one. Encourage them to stay away from social settings where they run the danger of relapsing, or accompany them to help keep them responsible.

Find your support system, practice self-care, and learn to let go

Discovering your personal healing source is vital. In addition to helping you, appropriate self-care and support will show your loved one in recovery how to operate in a healthy way. Family support groups offered by various inpatient or outpatient addiction treatment centers or addiction specialists, as well as mutual help groups like Al-Anon, Nar-Anon, and others, are great means of providing support for loved ones. The family should also consider to see a therapist who specializes in addiction and family dynamics.

CHAPTER VII

RELAPSE PREVENTION

Relapse prevention (RP) is a form of cognitive behavioral therapy that aims to help people sustain desired improvements by recognizing and addressing high-risk conditions for relapse. The goal of relapse prevention is to lessen the probability and impact of relapse after stopping or cutting back on the use of substance. Relapse Prevention focuses on preventing an initial lapse and offering lapse management in the case of a lapse so that subsequent relapses can be avoided. The first step in Relapse prevention is to identify situations that can pose a threat to an individual's effort to abstain from using the substance. It is important to identify the situations that are threatening and can act as a trigger for relapse. There can be both internal triggers or something that is in the individual's environment.

Strategies in Relapse Prevention

- Consume a balanced diet.
- Get adequate sleep.
- Engage in regular exercise and practice mindfulness
- Sign up for a self-help group, go to meetings on a regular basis, and take part in activities.
- Choose your environment and be in the company of upbeat and encouraging people.
- Be totally truthful with everyone in your support system, such as sponsors, relatives, physicians, therapists, and self-help group members.
- Regular counseling and therapy session.
- Take up a hobby or participate in activities of interest.
- Maintain a journal

